

Patient Info:

Patient Name: Thorne Alderwick (fictional synthetic patient)

Age: 39 years

Sex: Male

Laboratory Record Number: SYN-LAB-0007

Collection Date: 08 November 2026

Ordering Service: Endocrinology

Ordering Clinician: None listed

Clinical Indication: Evaluation of excessive thirst, frequent urination, blurred vision, and unexplained weight loss

Diagnosis:

Clinical Impression: Poorly controlled type 2 diabetes mellitus with dyslipidemia

Interpretation: Laboratory findings indicate chronic hyperglycemia with increased cardiovascular risk. Clinical correlation is recommended.

Report Status: Final

Disclaimer: This report is entirely fictional and synthetic for document-processing system testing only.

Medications:

Metformin Extended Release: 1000 mg orally twice daily with meals

Empagliflozin: 10 mg orally once daily

Atorvastatin: 20 mg orally at bedtime

Lab Results:

Test	Result	Reference Range
Fasting Plasma Glucose	246 mg/dL	70-99 mg/dL
Hemoglobin A1c	10.4 %	4.0-5.6 %
Random Blood Glucose	298 mg/dL	70-140 mg/dL
Urine Glucose	4+	Negative
Urine Ketones	Trace	Negative
Urine Microalbumin	124 mg/L	<30 mg/L
Serum Creatinine	1.02 mg/dL	0.70-1.30 mg/dL
Estimated Glomerular Filtration Rate (eGFR)	92 mL/min/1.73 m ²	>60 mL/min/1.73 m ²
Total Cholesterol	232 mg/dL	<200 mg/dL
LDL Cholesterol	152 mg/dL	<100 mg/dL
HDL Cholesterol	38 mg/dL	>40 mg/dL
Triglycerides	286 mg/dL	<150 mg/dL

Vitals:

Temperature: 36.8 degC

Blood Pressure: 148/92 mmHg

Heart Rate: 84 beats/minute

Respiratory Rate: 16 breaths/minute

Oxygen Saturation: 98% on room air

Height: 176 cm

Weight: 91.6 kg

Body Mass Index: 29.6 kg/m²

Comments:

The results demonstrate poorly controlled diabetes mellitus with persistent hyperglycemia, early diabetic kidney involvement, and mixed dyslipidemia. Close follow-up, lifestyle modification, and optimization of glucose-lowering therapy are recommended.

This laboratory report is entirely fictional and synthetic and is intended solely for testing document-processing and information-extraction systems.